جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نبويه عبد اللطيف جوده احمد
المحافظة : القليوبيه **الحي / المدينة :** شبين القناطر **العنوان :** مدينه نصر
رقم البطاقة : 24005041400423 **رقم التليفون :** 01113749255 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 04-05-1940 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نبويه عبد اللطيف جوده احمد
National ID: 24005041400423

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٤٠٠٥٠٤١٤٠٠٤٢٣ ٢٠٢٠/١٠

بدون عمل

انثى مسلمة أرملة

١ البطاقة سارية حتى ٢٠٢٧/١٠/٢٠





National ID Front & Back (same page)
[[DOCTYPE_ID_FRONT_BACK]]

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 3/9/2025 G

Name	نبوية عبد اللطيف	MRN	24005041400423
		ID	5255/2020
Age	80yrs	Phone	01137 49255
Menopausal status	postmenopausal	Marital status	
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	Remdt
CO morbidity	- grade I, II Nephropathy. - Rt MRM, Total hystrectomy		

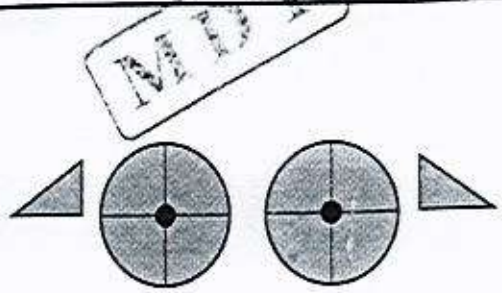
Clinical Data

BX: IDC II

ER:	PR:	HER 2:	Ki67:
8/8	8/8	-ve	25

Sonomammography:

Mets Workup:



TNM

Intially 2020
$cT_2 N_0$
upfront Rt MRM 2/2021

Recommendations: Post menopausal Rt breast Ca
$pT_{1\,1\text{-}5cm}\ pN_{0/16}\ M_0$
patient on adjuvant AI 5/2021 till Now.

طارق سعيد

MDT OUTCOME & PLAN

to start Adj AI till 5/2026
Prolia

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form [[DOCTYPE MDT_FORM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nabaweya Abd Al Latif Goda Ahmed	Date : 28/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 85 Year(s)
Medical ID: 5255/2020	Pat. ID: 13215

DIGITAL MAMMOGRAPHIC EXAMINATION AND COPMPLEMENTARY ULTRASOUND

- Status post right mastectomy.

Right Mastectomy Side:

- Clear operative bed.
- No solid or cystic lesions.
- No axillary lymphadenopathy.

Left Breast:

- ACR B
- No spiculated mass lesions or suspicious microcalcifications.
- No solid or cystic mass lesions.
- Normal duct system.
- Normal skin thickness and nipple/areola complex.
- No pathological axillary LNs.

OPINION:

- **Status post RIGHT MRM showing clear operative bed with no evidence of recurrent/residual suspicious lesions for follow up. BIRADS 2**

Signature Dr . Rehab El Badry

28/05/2025 4:23 PM

Sonography by Sonography / Ultrasound
[[DOCTYPE]] [[SONOGRAPHY]]

Dr.
Abdalla M. Khalil
Professor of Pathology
Faculty of Medicine
Kasr El-Aini

دكتور
عبدالله محمود خليل
أستاذ الباثولوجي – كلية طب قصر العيني
مستشار الباثولوجيا بمعهد السرطان القومي
عضو أكاديمية الباثولوجيا الدولية

Patient Name: نبويه عبد اللطيف جوده

Referred by Prof. Dr.: A. SHAWKY

Nature of the Specimen: RIGHT MRM

Clinical Diagnosis: MULTICENTERIC RIGHT BREAST CANCER

Receiving Date: Feb. / 2021 Code Number: A 55

PATHOLOGY REPORT

GROSS PATHOLOGY:

MRM specimen with skin surface area measured 25x15 cm including unremarkable nipple and areola. Serial cuts showed a fairly defined gray white mass measured 1.5x0.8 cm located 1.5 cm away from skin and 3 cm from least deep margin. As well as, a rubbery grayish white nodule measured 0.5x0.5 cm, located 1cm away from the mass

Also, there was an indurated cystified area measured 4x2 cm, located 3 cm away from the mass and 2 cm and 1.5 cm from skin and deep least margin respectively. Dissection of axilla revealed 16 nodules up to 2x1.5 cm.

HISTOLOGY:

Sections showed multiple invasive malignant neoplasms disposed in nests, sheets and groups of moderately anaplastic ductal cells with poor tubular formation associated with 20% DCIS component, surrounded by desmoplastic stromal response. The nipple, areola and resection margins were free. The axillary nodes showed reactive changes and were free of deposits.

DIAGNOSIS:

MULTICENTERIC INVASIVE DUCT CARCINOMA G2 WITH 20% DCIS, NO EVIDENT LYMPHOVASCULAR OR PERINEURAL INVASION, FREE NIPPLE, FREE MARGINS, FREE AXILLARY NODES (0/16) (T1, N0).

P.O.:

1- Detection of ER, PR, Her-2 and Ki67 is available on request.
2- 6 Slides enclosed.

A.M. Khalil, MD

عمارة الأعظمية: ٧٠ شارع المنيل - ميدان البيشا – المنيل - القاهرة تليفون: ٢٣٦٢٠١٠٧ - ٢٣٦٣٠٧١١ - ٢٣٦٣٠٧٣٥ - ٢٣٦٣٠٧٣٦ فاكس: ٢٣٦٢٠٣٦٩
El-Azamia Building:70 El-Manial St. El-Pasha SQ., El-Manial, Cairo Tel: (02)23620107-(02)23630711-(02)25320435-02)25320369

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number m5913

اسم المريض	نوبيه عبدالنظيف	النوع	Female
السن	80 Y	العنوان	القاهره
تاريخ اخذ العينة		تاريخ استلام العينة	03/11/2020 13:45
طالب التحليل	مستشفى الشيخ زايد ال نهيان	تاريخ التقرير	08/11/2020 10:06
عناية	مستشفى الشيخ زايد ال نهيان		

Primary Sample

Paraffin Block

Test

ER &PR & HER2 neu & Ki67 [ER&PR: Harvey scoring system -Her2neu : CAP scoring system]

Comment

Technique

sample recieved was treated by : - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (1E2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection Kit on ventana benchmark GX.
Sample recived was treated by : - CONFIRM anti Ki-67 (30-9) Rabbeit monoclonal primary antibody.

Result

ER strong nuclear staining of > 60 % of tumour cells (tiny fragment)
PR strong nuclear staining of > 60 % of tumour cells (tiny fragment)
HER2/neu protein over expression : NO membrane staining of tumor cells (tiny fragment).
Ki67 positive nuclear staining of 20 - 25 % of tumour cells (tiny fragment)

Referal Lab

Comment

Conclusion

ER POSITIVE (SCORE 8 / 8)
PR POSITIVE (SCORE 8 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE
#Ki67 THE PROLIFERATION INDEX OF THE TUMOUR IS 20 - 25 %

Pathologist د.نوبيه ماهر

Lab Director: د.نوبيه ماهر

وزارة الصحة
الإدارة المركزية للمعامل

19 El-sheikh Rehan Street, Abdeen, Cairo
Tel (155)7948544 -7950096 Fax (202)7948544
Mobile 01144297292
website http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook قسم الباثولوجى - الإدارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

ImmunohistochemistryImmunohistochemistry
[[DOCTYPE:IHC]][[DOCTYPE:IHC]]

Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 700

	Female	النوع	
	الناهره	العنوان	
	28/10/2020 12:36	تاريخ استلام العينه	
	03/11/2020 10:18	تاريخ التقرير	
	80 Y		سن
	مستشفى الشيخ زايد ال نهيان		
	مستشفى الشيخ زايد ال نهيان		

Primary sample

breast core biopsy

comment

Gross examination

tiny fragment of grey tan white tissue (T I)

Microscopic examination

Section examined revealed tiny fragment of fibrocollagenous tissue infiltrated by malignant growth display the morphology of infiltrating duct carcinoma

Referal Lab

Comment

Diagnosis

BREAST CORE : THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA (GI VERY TINY FRAGMENT)

Pathologist: Dr Jehan Aziz

Lab Director:

19 El-shekh Rehan Street, Abeidean, Cairo
Tel (165)7948544 -7950096 Fax (202)7948544
Mobile : 01144297292
website http://www.healthlabs.net
email pathology.chl@yahoo.com
facebook

* This Test is Not Accredited By EGAC Nor Recognized By ILA

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

AGOUZA SPINE CENTER

Phone: (___) ___-____ Fax: (___) ___-____

DXA Bone Densitometry Report: 11 March, 2023

Dear ??? ????,

Your patient nabwea abd alltef completed a BMD test on 11/03/2023 using the **Lunar Prodigy DXA System** (analysis version: 12.30) manufactured by **GE Healthcare**. The following summarizes the results of our evaluation.

PATIENT BIOGRAPHICAL:

Name:	nabwea abd alltef,				
Patient ID:	(not specified)	**Birth Date:**	04/05/1942	**Height:**	158.0 cm
Gender:	Female	**Exam Date:**	11/03/2023	**Weight:**	75.0 kg
Indications:		**Fractures:**		**Treatments:**	

ASSESSMENT:

The BMD measured at AP Spine L1-L4 is 1.232 g/cm² with a T-score of 0.3. This patient is considered normal according to World Health Organization (WHO) criteria. Fracture risk is low.

With a Z-Score of 2.3, this patient's BMD is well above adult normal range for this age.

World Health Organization (WHO) criteria for post-menopausal, Caucasian Women:

Normal	T-score at or above -1 SD
Osteopenia	T-score between -1 and -2.5 SD
Osteoporosis	T-score at or below -2.5 SD

RECOMMENDATIONS:

All patients should ensure an adequate intake of dietary calcium (1200 mg/d) and vitamin D (400-800 IU daily).

FOLLOW-UP:

People with diagnosed cases of osteoporosis or at high risk for fracture should have regular bone mineral density tests. For patients eligible for Medicare, routine testing is allowed once every 2 years. The testing frequency can be increased to one year for patients who have rapidly progressing disease, those who are receiving or discontinuing medical therapy to restore bone mass, or have additional risk factors.

Based on these results, a follow-up exam is recommended in March 2025

Sincerely,

(not specified)

Bone Scan Bone Scan
[[DOCTYPE BONE_SCAN]] SCAN]]

unar Prodigy - [nabwea abd alltef - 11/03/2023 12:16 PM]

File Measure Analyze Directory QA Composer Tools Window Help

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AP Spine | Right Femur | Right Forearm

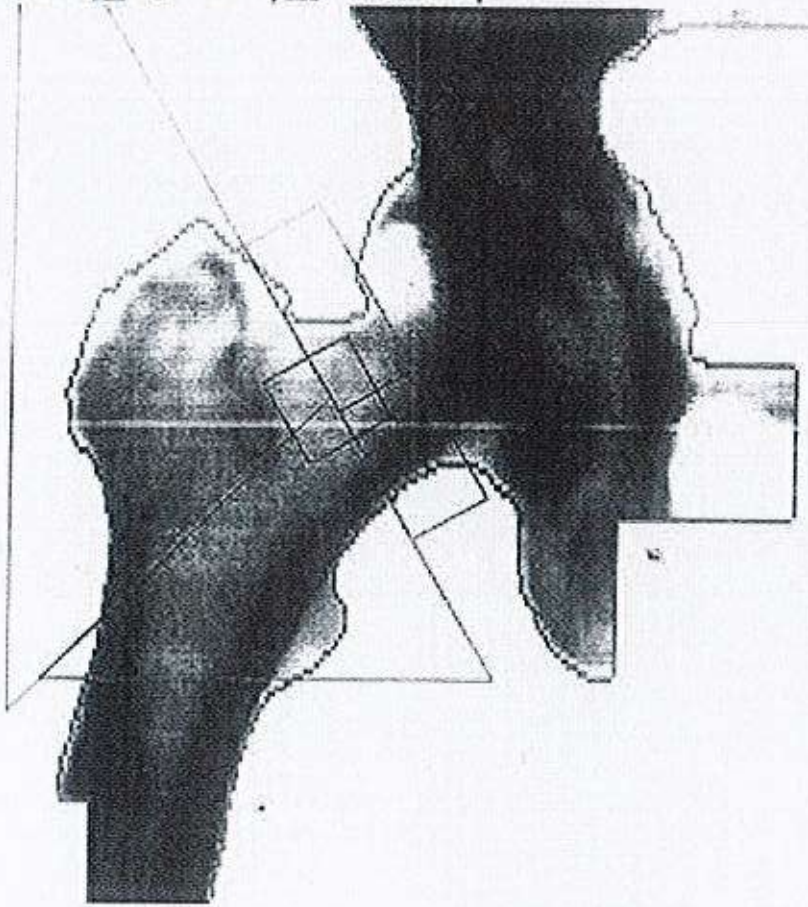
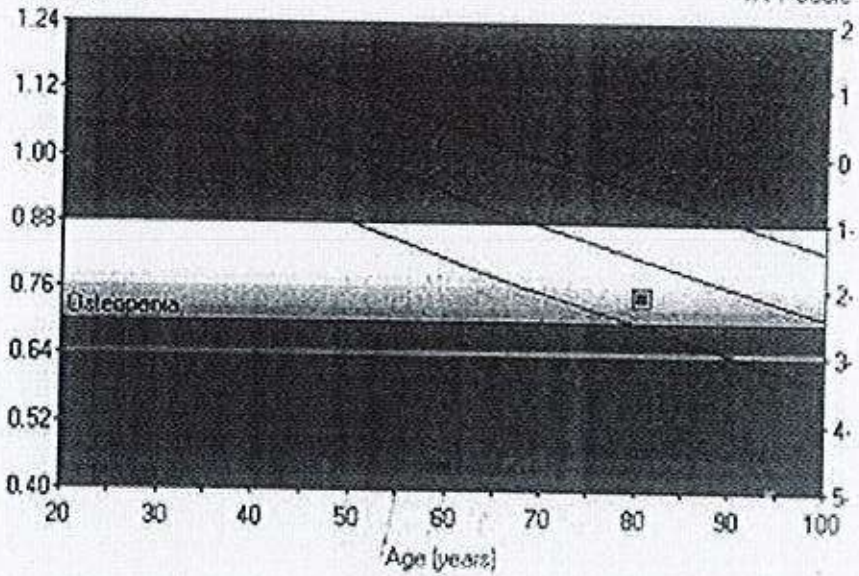


Image not for diagnosis

Densitometry | Composition | AHA | Information

Densitometry Reference: Total



Densitometry

Region	BMD (g/cm²)	BMC (g)	Area (cm²)	YA (%)	YA T-Score	AM (%)	AM Z-Score
Neck	0.637	3.13	4.92	65	2.9-	83	1.1-
Upper Neck	0.506	1.19	2.35	.	.	-	.
Lower Neck	0.756	1.95	2.57	.	.	.	.
Wards	0.428	1.15	2.69	47	3.7-	68	1.6-
Troch	0.648	11.13	17.17	82	1.3-	97	0.2-
Shaft	0.909	12.62	13.89	.	.	.	.
Total	0.747	28.89	38.97	75	2.1-	89	0.6-

Standard Results
Select region: up/down arrows
Results tab: left/right arrows
12:30 Image tab: tab/shift+tab

nabwea abd alltef
Born 04/05/1942 (80.8 years) 158.0 cm 75.0 kg Hispanic Female

Bone Scanner Scan
[DOCTYPE BONE SMART SCAN]

unar Prodigy - [nabwea abd alliet - 11/03/2023 12:16 PM]

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sure Analyze Directory QA Imaging ROIs Points Copy Reports Close

AP Spine | Right Femur | Right Forearm

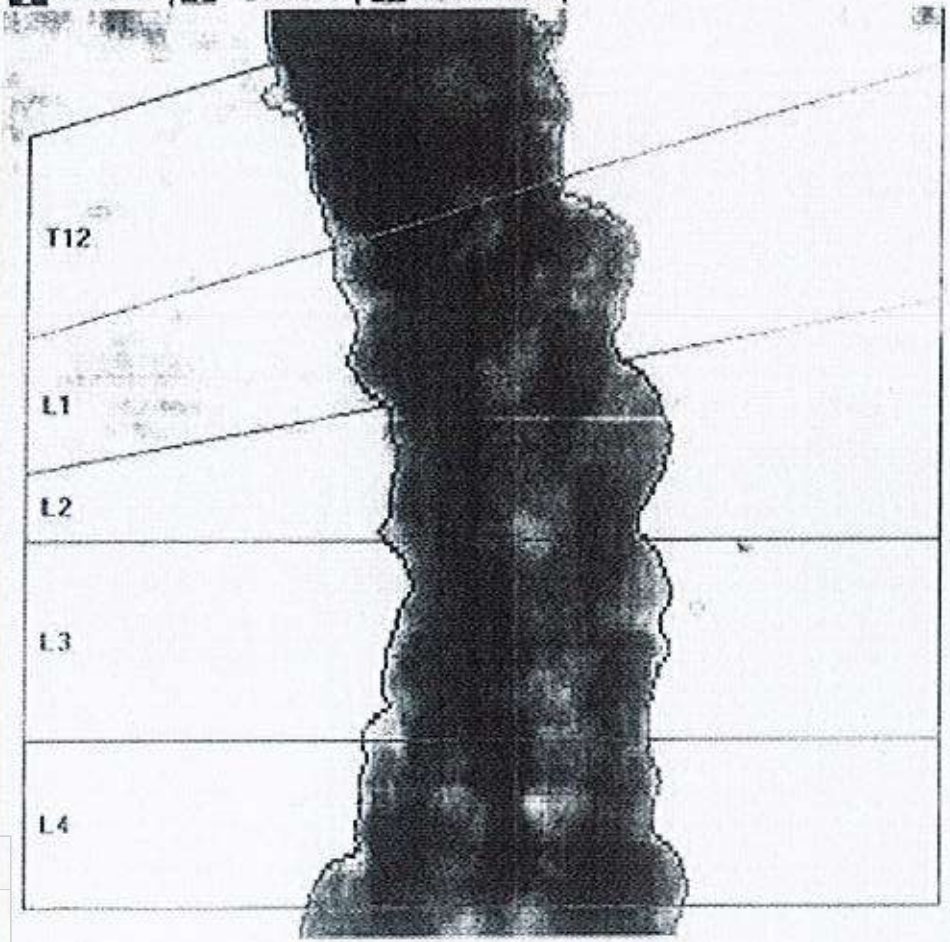
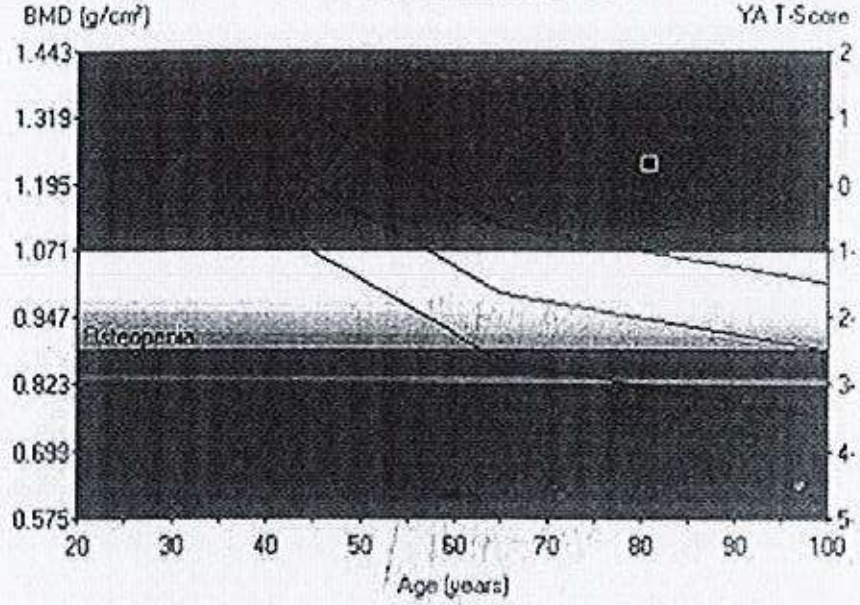


Image not for diagnosis

Densitometry | Composition | Information

Densitometry Reference: L1-L4



Densitometry

Region	BMD (g/cm²)	BMC (g)	Area (cm²)	YA (%)	YA T-Score	AM (%)	AM Z-Score
T12	1.454	24.91	17.14	-	-	-	-
L1	1.330	22.25	16.72	116	1.5	147	3.5
L2	1.049	15.14	14.43	87	1.3-	108	0.7
L3	1.247	23.13	18.55	102	0.2	128	2.2
L4	1.272	23.27	18.29	104	0.4	130	2.4
L1-L2	1.200	37.39	31.16	102	0.2	128	2.2
L1-L3	1.217	60.51	49.71	103	0.3	129	2.3
L1-L4	1.232	83.78	68.00	104	0.5	129	2.3
L2-L3	1.160	38.26	32.98	96	0.4-	119	1.5
L2-L4	1.200	61.53	51.28	99	0.1-	123	1.9
L3-L4	1.259	46.39	36.85	103	0.3	129	2.3

Standard Results
Select region up/down arrows
Results tab left/right arrows

nabwea abd alliet
Born 04/05/1942 (80.8 years) 158.0 cm 75.0 kg Hispanic Female

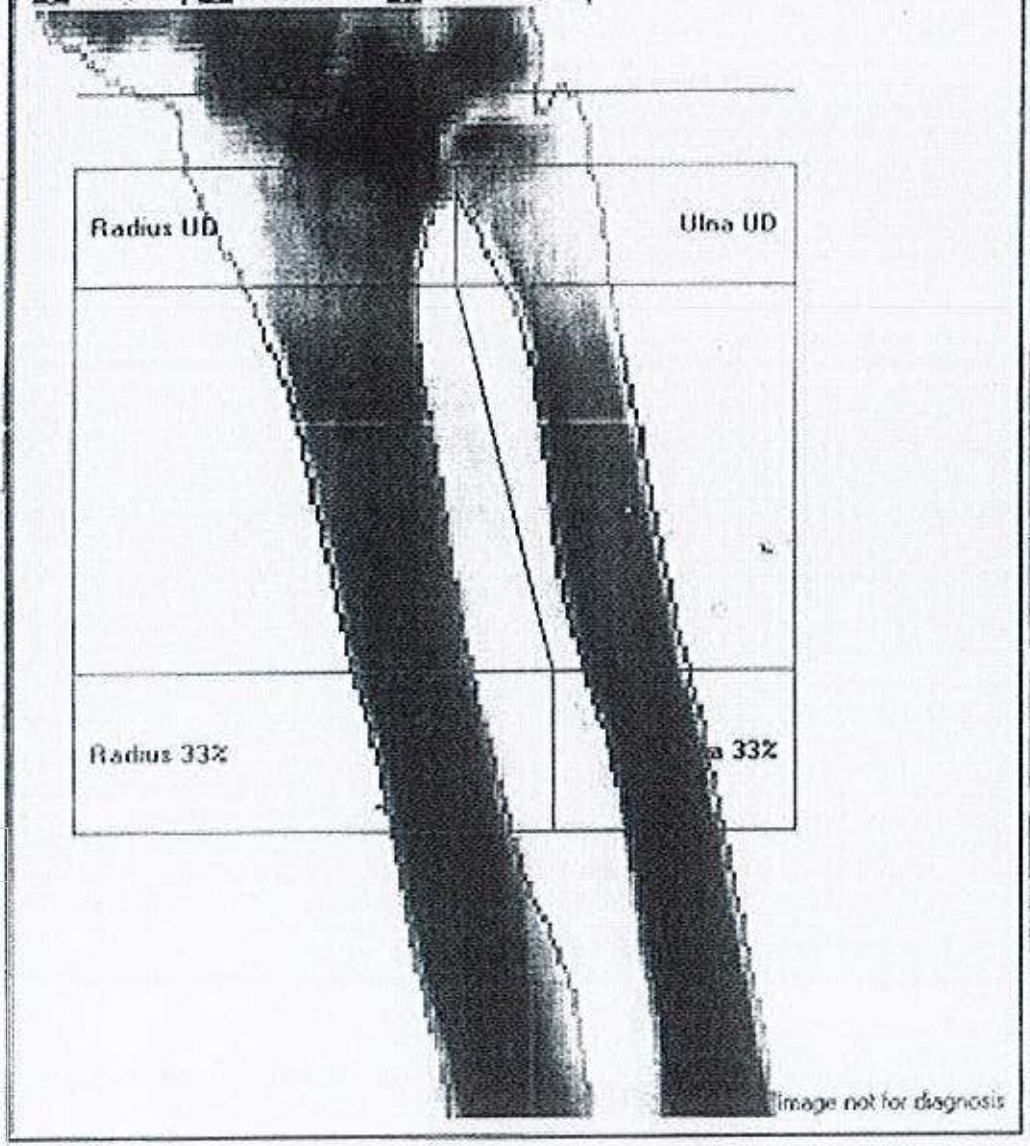
Property of Bone Densitometry Technologies
DO NOT REPRODUCE SCAN!

unar Prodigy - [nabwea abd alltef - 11/03/2023 12:16 PM]

File Measure Analyze Directory QA Composer Tools Window Help

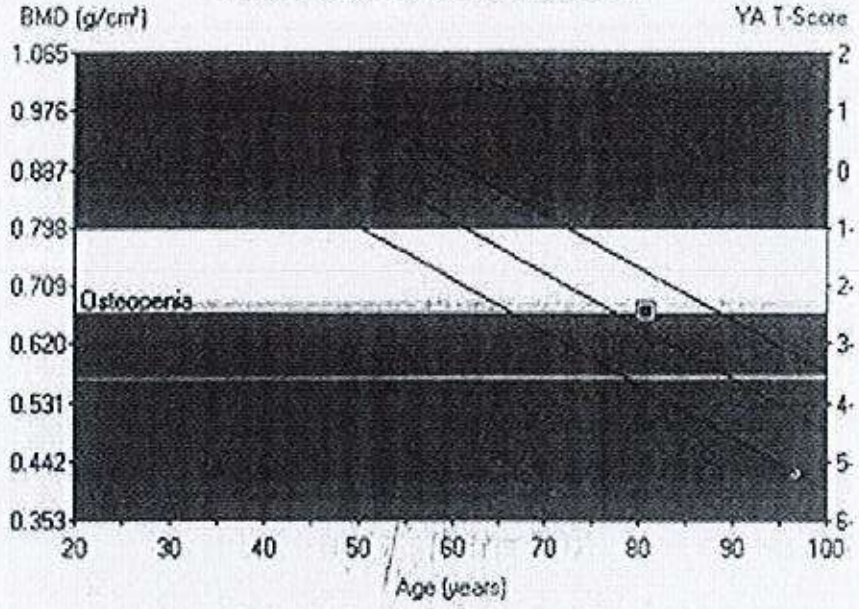
asure | Analyze | Directory | QA | Imaging | ROIs | Points | Copy | Reports | Close

AP Spine | Right Femur | Right Forearm



Densitometry | Information

Densitometry Reference: Radius 33%



Densitometry

Region	BMD (g/cm²)	BMC (g)	Area (cm²)	YA (%)	YA T-Score	AM (%)	AM Z-Score
Radius UD	0.362	1.43	3.95	77	2.3	106	0.5
Ulna UD	0.259	0.58	2.29	.	.	.	.
Radius 33%	0.667	1.73	2.59	75	2.5	104	0.3
Ulna 33%	0.630	1.45	2.31	.	.	.	.
Both UD	0.324	2.02	6.24	.	.	.	.
Both 33%	0.650	3.25	5.01	.	.	.	.
Radius Total	0.499	7.06	14.14	73	3.0	97	0.2
Ulna Total	0.447	4.60	10.30	.	.	.	.
Both Total	0.477	11.66	24.44	.	.	.	.

nabwea abd alltef

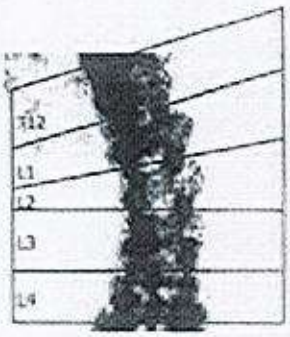
Born 04/05/1942 (80.8 years) 158.0 cm 75.0 kg Hispanic Female

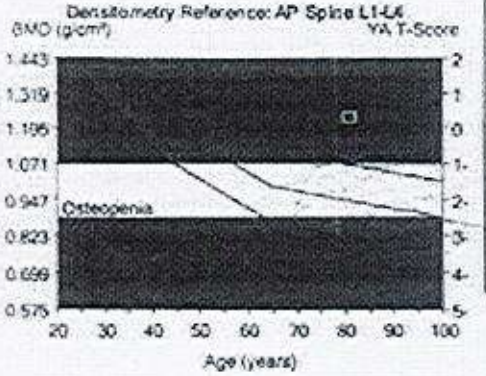
12:36

Bone Scan [DOCTYPE BONE SCAN] SCAN]

AGOUZA SPINE CENTER

Patient:	nabwea abd alitef,	**Facility ID:**	
Birth Date:	04/05/1942 80.8 years	**Patient:**	nabwea abd alitef,
Height / Weight:	158.0 cm 75.0 kg	**Measured:**	11/03/2023 12:13:53 PM (12.30)
Sex / Ethnic:	Female	**Analyzed:**	11/03/2023 12:16:54 PM (12.30)

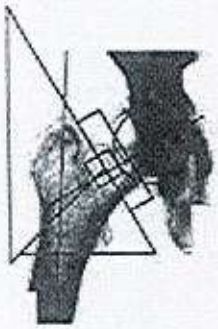


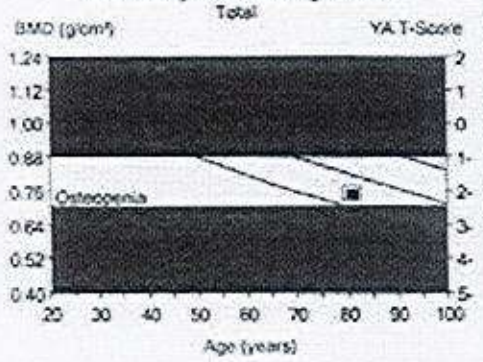


Region	BMD (g/cm²)	Young-Adult (%)	Young-Adult T-Score	Age-Matched (%)	Age-Matched Z-Sco
L1	1.330	116	1.5	147	3.5
L2	1.049	87	1.3-	108	0.7
L3	1.247	102	0.2	128	2.2
L4	1.272	104	0.4	130	2.4
L1-L2	1.200	102	0.2	128	2.2
L1-L3	1.217	103	0.3	129	2.3
L1-L4	1.232	103	0.3	129	2.3
L2-L3	1.160	96	0.4-	119	1.5
L3-L4	1.259	103	0.3	129	2.3

Matched for Age, Weight (females 25-100 kg), Ethn
Spain (ages 20-40) AP Spine Reference Population (v11
Statistically 68% of repeat scans fall within 1SD (± 0.010 g/cm² for AP Spine L1-4

Image not for diagnosis

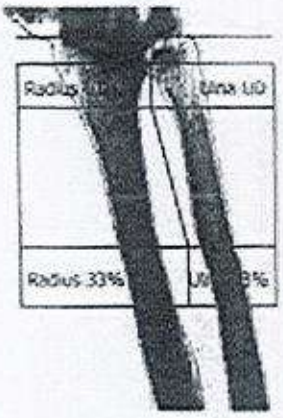


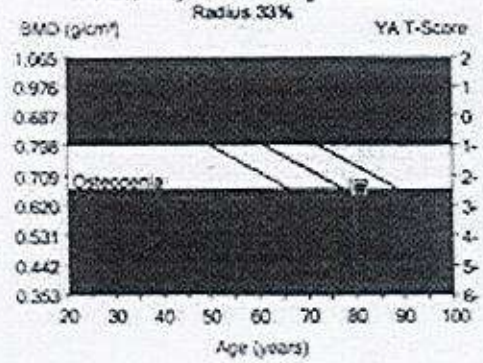


Region	BMD (g/cm²)	Young-Adult (%)	Young-Adult T-Score	Age-Matched (%)	Age-Matched Z-Sco
Neck	0.637	65	2.9-	83	1.1-
Upper Neck	0.506	-	-	-	-
Lower Neck	0.756	-	-	-	-
Wards	0.428	47	3.7-	68	1.6-
Troch	0.648	82	1.3-	97	0.2-
Shaft	0.909	-	-	-	-
Total	0.747	75	2.1-	92	0.6-

Matched for Age, Weight (females 25-100 kg), Ethn
Spain (ages 20-40) Femur Reference Population (v11
Statistically 68% of repeat scans fall within 1SD (± 0.012 g/cm² for Right Femur Tot

Image not for diagnosis





Region	BMD (g/cm²)	Young-Adult (%)	Young-Adult T-Score	Age-Matched (%)	Age-Matched Z-Sco
Radius UD	0.362	77	2.3-	106	0.5
Ulna UD	0.259	-	-	-	-
Radius 33%	0.667	75	2.5-	104	0.3
Ulna 33%	0.630	-	-	-	-
Both UD	0.324	-	-	-	-
Both 33%	0.650	-	-	-	-
Radius Total	0.499	73	3.0-	97	0.2-
Ulna Total	0.447	-	-	-	-
Both Total	0.477	-	-	-	-

Lunar calibration is ok
Matched for Age, Ethn
Spain (ages 20-40) Forearm Reference Population (v11
Statistically 68% of repeat scans fall within 1SD (± 0.020 g/cm² for Right Forearm Radius 33%

Image not for diagnosis